

25 mm



Stadeltime Film-Coated Tablet 5mg

1. **Name of the medicinal product**
Stadeltime Film-Coated Tablet 5mg
2. **Special notice and recommendation**
Keep out of reach of children
Read the package insert carefully before use
3. **Composition**
Each film-coated tablet contains:
Levocetirizine dihydrochloride.....5 mg
4. **Pharmaceutical form**
Film-coated tablet.
White, caplet-shaped, film-coated tablet, biconvex, engraved with "5" on one side and plain on the other side.
5. **Indications**
Levocetirizine is indicated for the symptomatic treatment of allergic rhinitis (including persistent allergic rhinitis) and chronic idiopathic urticaria.
6. **Administration and dosage**
Administration
Oral use. Swallow whole with liquid and may be taken with or without food. It is recommended to take the daily dose in one single intake.
Dosage
Adults and adolescents 12 years and above
The daily recommended dose is 5 mg (one film-coated tablet).
Elderly
Adjustment of the dose is recommended in elderly patients with moderate to severe renal impairment (see Renal impairment below).
Renal impairment
The dosing intervals must be individualized according to renal function. Refer to the following table and adjust the dose as indicated. To use this dosing table, an estimate of the patient's creatinine clearance (CL_{Cr}) in ml/min is needed. The CL_{Cr} (ml/min) may be estimated from serum creatinine (mg/dl) determination using the following formula:
$$CL_{Cr} = \frac{[140 - \text{age}(\text{years})] \times \text{weight}(\text{kg})}{72 \times \text{serum creatinine}(\text{mg/dl})} \quad (\times 0.85 \text{ for women})$$

Dosing adjustments for patients with impaired renal function:

Group	Creatinine clearance (ml/min)	Dosage and frequency
Normal	≥ 80	1 tablet once daily
Mild	50 - 79	1 tablet once daily
Moderate	30 - 49	1 tablet once every 2 days
Severe	< 30	1 tablet once every 3 days
End-stage renal disease – Patients undergoing dialysis	< 10	Contra-indicated

In paediatric patients suffering from renal impairment, the dose will have to be adjusted on an individual basis taking into account the renal clearance of the patient and his body weight. There are no specific data for children with renal impairment.
Hepatic impairment
No dose adjustment is needed in patients with solely hepatic impairment. In patients with hepatic impairment and renal impairment adjustment of the dose is recommended (see Renal impairment above).
Paediatric population
Children aged 6 to 12 years: The daily recommended dose is 5 mg (one film coated tablet).
Children aged 2 to 6 years: No adjusted dosage is possible with the film-coated tablet formulation. It is recommended to use a paediatric formulation of levocetirizine.
Duration of use
Intermittent allergic rhinitis (symptoms experienced for less than four days a week or for less than four weeks a year) has to be treated according to the disease and its history; it can be stopped once the symptoms have disappeared and can be restarted again when symptoms reappear. In case of persistent allergic rhinitis (symptoms experienced for more than four days a week or for more than four weeks a year), continuous therapy can be proposed to the patient during the period of exposure to allergens.
There is clinical experience with the use of levocetirizine for treatment periods of at least 6 months. In chronic urticaria and chronic allergic rhinitis, there is clinical experience of use of cetirizine (racemate) for up to one year.
7. **Contraindications**
Hypersensitivity to the active substance, to cetirizine, to hydroxyzine, to any of the excipients in the formula.
Severe renal impairment at less than 10ml/min creatinine clearance.

8. Special warnings and precautions for use

- Precaution is recommended with concurrent intake of alcohol.
 - Caution should be taken in patients with predisposing factors of urinary retention (e.g. spinal cord lesion, prostatic hyperplasia) as levocetirizine may increase the risk of urinary retention.
 - Caution should be taken in patients with epilepsy and patients at risk of convulsion as levocetirizine may cause seizure aggravation.
 - Response to allergy skin tests are inhibited by antihistamines and a wash-out period (of 3 days) is required before performing them.
 - Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.
 - Pruritus may occur when levocetirizine is stopped even if those symptoms were not present before treatment initiation. The symptoms may resolve spontaneously. In some cases, the symptoms may be intense and may require treatment to be restarted. The symptoms should resolve when the treatment is restarted.
- Paediatric population:

The use of the film-coated tablet formulation is not recommended in children aged less than 6 years since this formulation does not allow for appropriate dose adaptation. It is recommended to use a paediatric formulation of levocetirizine.

9. Pregnancy and lactation

Pregnancy

The use of levocetirizine may be considered during pregnancy, if necessary. There are no or limited amount of data (less than 300 pregnancy outcomes) from the use of levocetirizine in pregnant women. However, for cetirizine, the racemate of levocetirizine, a large amount of data (more than 1000 pregnancy outcomes) on pregnant women indicate no malformative or feto/neonatal toxicity.

Lactation

Cetirizine, the racemate of levocetirizine, has been shown to be excreted in human. Therefore, the excretion of levocetirizine in human milk is likely. Adverse reactions associated with levocetirizine may be observed in breastfed infants. Therefore, caution should be exercised when prescribing levocetirizine to lactating women.

10. Effects on ability to drive and use machines

- It has been revealed that no evidence that levocetirizine at the recommended dose impairs mental alertness, reactivity or the ability to drive.
- Nevertheless, some patients could experience somnolence, fatigue and asthenia under therapy with levocetirizine. Therefore, patients intending to drive, engage in potentially hazardous activities or operate machinery should take their response to the medicinal product into account.

11. Drug interactions

- No interaction studies have been performed with levocetirizine (including no studies with CYP3A4 inducers); studies with the racemate compound cetirizine demonstrated that there were no clinically relevant adverse interactions (with antipyrine, pseudoephedrine, cimetidine, ketoconazole, erythromycin, azithromycin, glipizide and diazepam). A small decrease in the clearance of cetirizine (16%) was observed with theophylline (400 mg once a day); while the disposition of theophylline was not altered by concomitant cetirizine administration.
- The extent of exposure to cetirizine was increased by about 40% while the disposition of ritonavir was slightly altered (-11%) further to concomitant cetirizine administration.
- The extent of absorption of levocetirizine is not reduced with food, although the rate of absorption is decreased.
- In sensitive patients, the concurrent administration of cetirizine or levocetirizine and alcohol or other CNS depressants may cause additional reductions in alertness and impairment of performance.

12. Adverse reactions

Nervous system disorders

Common: headache, somnolence

Gastrointestinal disorders

Common: dry mouth

Uncommon: abdominal pain

General disorders and administration site conditions

Common: fatigue

Uncommon: asthenia

The incidence of sedating adverse drug reactions such as somnolence, fatigue, and asthenia was altogether more common (8.1 %) under levocetirizine 5 mg than under placebo (3.1%).

Post Marketing Data

Immune system disorders

Not known: hypersensitivity including anaphylaxis

Metabolism and nutrition disorders

Not known: increased weight, increased appetite

27.5 mm



Psychiatric disorders

Not known: aggression, agitation, hallucination, depression, insomnia, suicidal ideation, nightmares

Nervous system disorders

Not known: convulsions, paraesthesia, dizziness, syncope, tremor, dysgeusia

Eyes disorders

Not known: visual disturbances, blurred vision, oculogyration

Ear and labyrinth disorders

Not known: vertigo

Cardiac disorders

Not known: palpitations, tachycardia

Respiratory, thoracic, and mediastinal disorders

Not known: dyspnoea

Gastrointestinal disorders

Not known: nausea, vomiting, diarrhoea

Hepatobiliary disorders

Not known: hepatitis, abnormal liver function test

Skin and subcutaneous tissue disorders

Not known: angioneurotic oedema, fixed drug eruption, pruritus, rash, urticaria

Musculoskeletal and connective tissues disorders

Not known: myalgia, arthralgia

Renal and urinary disorders

Not known: dysuria, urinary retention

General disorders and administration site conditions

Not known: oedema

Skin reactions occurring after discontinuation of levocetirizine

After levocetirizine discontinuation, pruritus has been reported.

13. Overdosage and managementSymptoms

Symptoms of overdose may include drowsiness in adults. In children, agitation and restlessness may initially occur, followed by drowsiness.

Management

- There is no known specific antidote to levocetirizine.
- Should overdose occur, symptomatic or supportive treatment is recommended.
- Gastric lavage may be considered shortly after ingestion of the drug.
- Levocetirizine is not effectively removed by haemodialysis.

14. Pharmacodynamic properties

- Pharmacotherapeutic group: antihistamine for systemic use, piperazine derivatives
- ATC code: R06A E09.
- Levocetirizine, the (R) enantiomer of cetirizine, is a potent and selective antagonist of peripheral H1-receptors.
- It is revealed that levocetirizine has high affinity for human H1-receptors ($K_i = 3.2 \text{ nmol/l}$). Levocetirizine has an affinity 2-fold higher than that of cetirizine ($K_i = 6.3 \text{ nmol/l}$). Levocetirizine dissociates from H1-receptors with a half-life of $115 \pm 38 \text{ min}$.
- After single administration, levocetirizine shows a receptor occupancy of 90% at 4 hours and 57% at 24 hours.

15. Pharmacokinetic properties

The pharmacokinetics of levocetirizine are linear with dose- and time-independent with low inter-subject variability. The pharmacokinetic profile is the same when given as the single enantiomer or when given as cetirizine. No chiral inversion occurs during the process of absorption and elimination.

Absorption

Levocetirizine is rapidly and extensively absorbed following oral administration. In adults, peak plasma concentrations are achieved 0.9 hour after dosing. Steady state is achieved after two days. Peak concentrations are typically 270 ng/ml and 308 ng/ml following a single and a repeated 5 mg once daily dose, respectively. The extent of absorption is dose-independent and is not altered by food, but the peak concentration is reduced and delayed.

Distribution

No tissue distribution data are available in humans, neither concerning the passage of levocetirizine through the blood-brain-barrier. In rats and dogs, the highest tissue levels are found in liver and kidneys, the lowest in the CNS compartment.

In humans, levocetirizine is 90% bound to plasma proteins. The distribution of levocetirizine is restrictive, as the volume of distribution is 0.4 l/kg.

Biotransformation

The extent of metabolism of levocetirizine in humans is less than 14% of the dose and therefore differences resulting from genetic polymorphism or concomitant intake of enzyme inhibitors are expected to be negligible.

Metabolic pathways include aromatic oxidation, N- and O- dealkylation and taurine conjugation. Dealkylation pathways are primarily mediated by CYP 3A4 while aromatic oxidation involved multiple and/or unidentified CYP isoforms. Levocetirizine had no effect on the activities of CYP isoenzymes 1A2, 2C9, 2C19, 2D6, 2E1 and 3A4 at concentrations well above peak concentrations achieved following a 5 mg oral dose.

Due to its low metabolism and absence of metabolic inhibition potential, the interaction of levocetirizine with other substances, or vice-versa, is unlikely.

Elimination

The plasma half-life in adults is 7.9 ± 1.9 hours. The half-life is shorter in small children.

The mean apparent total body clearance in adults is 0.63 ml/min/kg. The major route of excretion of levocetirizine and metabolites is via urine, accounting for a mean of 85.4% of the dose. Excretion via faeces accounts for only 12.9% of the dose. Levocetirizine is excreted both by glomerular filtration and active tubular secretion.

Special populationRenal impairment

The apparent body clearance of levocetirizine is correlated to the creatinine clearance. It is therefore recommended to adjust the dosing intervals of levocetirizine, based on creatinine clearance in patients with moderate and severe renal impairment. In anuric end stage renal disease patients, the total body clearance is decreased by approximately 80% when compared to normal patients. The amount of levocetirizine removed during a standard 4-hour hemodialysis procedure was < 10%.

Paediatric population

Oral administration of a single dose of 5 mg levocetirizine in children age 6 to 11 years with body weight ranging between 20 and 40 kg show that C_{max} and AUC values are about 2-fold greater than that reported in healthy adults. The mean C_{max} was 450 ng/ml, occurring at a mean time of 1.2 hours, weight-normalized, total body clearance was 30% greater, and the elimination half-life 24% shorter in this paediatric population than in adults.

Elderly

Limited pharmacokinetic data are available in elderly. The total body clearance was approximately 33% lower compared to that in younger adults. The disposition of racemic cetirizine has been shown to be dependent on renal function rather than on age. This finding would also be applicable for levocetirizine, as levocetirizine and cetirizine are both predominantly excreted in urine. Therefore, the levocetirizine dose should be adjusted in accordance with renal function in elderly patients.

Gender

Pharmacokinetic results for the patients (both men and women) were evaluated for potential effect of gender. The half-life was slightly shorter in women (7.08 hours $\pm$ 1.72 hours) than in men (8.62 hours $\pm$ 1.84 hours); however, the body weight-adjusted oral clearance in women (0.67 $\pm$ 0.16 ml/min/kg) appears to be comparable to that in men (0.59 $\pm$ 0.12 ml/min/kg). The same daily doses and dosing intervals are applicable for men and women with normal renal function.

Race

The effect of race on levocetirizine has not been studied. As levocetirizine is primarily renally excreted, and there are no important racial differences in creatinine clearance, pharmacokinetic characteristics of levocetirizine are not expected to be different across races. No race-related differences in the kinetics of racemic cetirizine have been observed.

Hepatic impairment

The pharmacokinetics of levocetirizine in the hepatically impaired have not been tested. Patients with chronic liver diseases (hepatocellular, cholestatic, and biliary cirrhosis) given 10 or 20 mg of the racemic compound cetirizine as a single dose had a 50% increase in half-life along with a 40% decrease in clearance compared to those who are healthy.

16. Packaging

Box of 5 blisters. Blister of 10 tablets.

17. Storage condition, shelf-life

Do not store above 30°C.

The expiry date of this pack is printed on the box. Do not use this pack after this date.

18. Name, address of manufacturer

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Date of revision: December 25th, 2023

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